



CNL-664A Topic 4 Biopsychosocial Assessment Template

Client's Name: Isaiah

Date: 6/6/2026

DOB: 6/19/1995

Age: 30

Start Time: Enter the start time here.

End Time: Enter the end time here.

Identifying Information:

Isaiah is a 30 year old male. He works as a delivery driver for a company and has been in therapy multiple times before, some worked well and others did not. He has 3 kids, two of which he has no contact with. He currently lives with his girlfriend and her family and has his daughter Kiana with him.

Presenting Problem/Chief Complaint:

Suffered with depression and anxiety for most of his life. Had a traumatic experience leaving the military. His relationship with his girlfriend is starting to suffer similar to his previous marriage. This has been a problem for most of his life, depressed while in the military. It has been a year that he has been dealing with severe depression. Claims it has been harder the last few months.

Substance Use History:

No reported substance abuse history.

Addictions (i.e., gambling, pornography, video gaming):

No reported addictions.

Medical History/Mental Health History/Hospitalizations:

Client reported complications at birth, not sure exactly what happened but it was life threatening. He has also experienced some significant weight loss (perhaps from the job, also not eating very much) (Around 30 pounds in 5 months). He spent a few months at North Kansas City Valor Mental hospital for suicidal ideation.

Abuse/Trauma History:

No reported abuse, however, there is some history of trauma surrounding his experience in the military. In addition, there is some trauma connected to his previous marriage in which case he is no longer able to be in contact of his two children.

Social History and Resources:

Mom is the main support line. Good friend Aedan, have an issue keeping in contact with people, lives in Illinois. Girlfriend's family is awkward (operate differently, disagree with them)

Legal History:

The only reported legal issues have been divorce and custody battles, including disputes regarding child support.

Educational History:

Started school at Utah Tech, but dropped out due to financial hardship. Has the desire to be an elementary school teacher. Did fairly well while in high school but didn't go into detail.

Family History:

Very close relationship with mom, been like that his whole life. No real relationship with his biological dad. Dad was around till he was 9, they broke up, lived with dad for a while in England, then he left shortly after and he is unsure why. Claimed dad probably didn't care. Saw him in 2012, but has not seen him since. Older sister (Tiara, 32) No relationship, she is the black sheep of the family, lives in St. Louis . Never really close growing up.
Younger brother (Devantre, 25) Somewhat close but they don't talk much, in the military, lives in England. Half-brother (not sure what his name is no relationship).
Got married in 2020 to Corrianne, she had two kids prior, adopted oldest daughter (Grace), then had a daughter in 2021 (Haven). As of now, no contact with them. Facetimed a few times.
Corrianne had a son Thomas, dad is still around.
Has a daughter (Kiana, 1) with Kenzie. Good relationship with Kiana.

Cultural Factors:

Atheist, but sometimes believes that there could be something more. His girlfriend's family is very religious, very active members of the LDS church.

Resources, Strengths, and Weaknesses:

He is good at multitasking, he is able to manage a multitude of things at once. He is trustworthy, reliable, and caring. He struggles with change and being out of his comfort zone.

Case Conceptualization (Conceptualize the case using your preferred theoretical orientation):

Isaiah is a 30-year-old male presenting with chronic depression and anxiety. From a cognitive-behavioral and trauma-informed perspective, his symptoms appear to be influenced by a combination of early attachment disruptions, military-related trauma, as well as past and current relational stressors. Isaiah's history of an inconsistent paternal relationship and perceived abandonment may have contributed to core beliefs related to worthlessness, rejection, and instability in relationships. These beliefs appear to be reinforced by his current relational difficulties with his girlfriend and the loss of contact with two of his children following his previous marriage.

Additionally, Isaiah's traumatic transition out of the military may contribute to maladaptive cognitive patterns, emotional dysregulation, and possible unresolved trauma symptoms. His depressive symptoms, including significant weight loss, social withdrawal, and prior suicidal ideation requiring hospitalization, suggest persistent negative thought patterns and behavioral disengagement. Isaiah also demonstrates difficulty maintaining social connections, which further perpetuates isolation and depressive symptoms. Despite these challenges, protective factors include a strong relationship with his mother, a positive bond with his daughter Kiana, and personal strengths such as reliability and caring, which can be leveraged in treatment.

Clinical Justification:

Isaiah's presentation is consistent with symptoms of a depressive disorder, including prolonged depressed mood, significant weight loss, social withdrawal, and a history of suicidal ideation requiring hospitalization. The chronic nature of his depression, along with reported anxiety and functional impairment in relationships and daily life, supports the need for ongoing clinical intervention. His trauma history related to military experiences and relational losses further complicates his clinical picture and may contribute to both depressive and anxiety symptoms. From a clinical standpoint, treatment is justified due to the severity and duration of symptoms, as well as risk factors such as prior hospitalization for suicidal ideation. A CBT and trauma-informed approach is appropriate to address maladaptive cognitions, improve emotional regulation, and process unresolved trauma. Additionally, relational interventions may be beneficial in addressing patterns that contribute to interpersonal conflict and disconnection. Isaiah's identified strengths and support systems provide a foundation for treatment engagement and prognosis, though his difficulty with change and maintaining relationships may present barriers that require ongoing clinical attention.

Initial Diagnosis (Current ICD and DSM):

Principal Diagnosis:	<i>ICD Code:</i>	<i>DSM Disorder:</i>	<i>Subtypes:</i>	<i>Specifiers:</i>
	F33.1	Major Depressive Disorder		Recurrent, Moderate
Provisional Diagnosis:	<i>ICD Code:</i>	<i>DSM Disorder:</i>	<i>Subtypes:</i>	<i>Specifiers:</i>

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Initial Treatment Goals Informed by Theoretical Orientation (SMART Goal Format):

Goal # 1: Alleviate depressive symptoms and return to previous level of effective functioning.		
Objectives:	Interventions:	Target Date:
1. Work cooperatively with the therapist toward agreed-upon therapeutic goals while being as open and honest as comfort and trust allow.	Establish rapport with the client toward building a strong therapeutic alliance; convey caring, support, warmth, and empathy; provide nonjudgmental support and develop a level of trust with the client toward feeling safe to discuss depression and its impact on his/her/their life.	Every Week
2. Increasingly verbalize hopeful and positive statements regarding self, others, and the future.	Assign the client to write at least one positive affirmation statement daily regarding themselves and the future (or supplement with "Positive Self-Talk" in the Adult Psychotherapy Homework Planner by Jongsma).	Every Week
Goal # 2: Recognize, accept, and cope with feelings of depression.		
Objectives:	Interventions:	Target Date:
1. Verbalize an accurate understanding of depression.	Consistent with the treatment model, discuss how cognitive, behavioral, interpersonal, and/or other factors (e.g., family history) contribute to depression.	Every Week

2. Learn and implement behavioral strategies to overcome depression.	Assist the client in developing skills that increase the likelihood of deriving pleasure and meaning from behavioral activation (e.g., assertiveness skills, developing an exercise plan, less internal/more external focus, increased social involvement); reinforce success; problem-solve obstacles toward sustained, rewarding activation.	Every Week
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Student Clinician's Name: Kolby J. Ray

Date: 3/19/2026

Reflection 08/25/2026

Throughout my development of biopsychosocial assessments, I have grown in my ability to approach each client with greater amiability. Early on, I viewed the biopsychosocial as primarily an assessment tool that required me to gather specific information efficiently. As I gained more experience, I began to recognize how important my demeanor and ability to build rapport are to the quality of the information a client is willing to share.

I have learned that being approachable, warm, and genuinely interested in the client's experience can make difficult or personal questions feel less intimidating. Rather than simply moving through a checklist, I have become more intentional about using active listening, appropriate empathy, and a conversational tone. This has helped me create an environment where clients feel more comfortable discussing sensitive areas of their lives.

I still have room to grow, particularly in balancing amiability with the need to gather comprehensive information within a limited amount of time. However, I feel much more confident in allowing the assessment to feel like a conversation rather than an interrogation. Developing this skill has reminded me that the therapeutic relationship begins from the very first interaction, and even an assessment can be an opportunity to demonstrate acceptance, respect, and genuine care for the client.